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The Parent, the Picture, and the Pandemic: How One Mother's Viral Photo Captured the Need for School Choice.

By March 27, 2020, all but one U.S. public school district closed to in-person learning to slow the spread of COVID (Zviedrite et al. 4). Almost immediately, the effects of the school closures were felt by parents. In August of 2020, Jana Coombs took a picture of her son frustrated to tears with his remote learning experience (Ross and Thomas). In the picture, a five-year-old boy sits slumped back in his chair crying, his shirt is pulled up to his face to cover the tears as his Kindergarten class is conducted on the laptop in front him (Ross and Thomas). That photo went viral. In an interview with Kaitlyn Ross of *11 Alive*, Coombs said, "I just took that picture because I wanted people to see reality." The reality for some children in the United States is that COVID-19 mitigation strategies like school closures have been a source of emotional disturbance and learning loss. However, parents with the means to affect change saw this happening and were able to do something about it: they looked at their five-year-old, saw the tears, and said to themselves "I'm not doing this anymore." Instead, they opted for homeschooling, private schools, or moved. But not all families had that same privilege. Although each family has contended with varying degrees of difficulty during the pandemic, it was low-income and working-class communities who were hit the hardest, resulting in inequity that highlights the need for school choice for all Americans.

Americans have a long history of self-correcting. "The great privilege of the Americans," observed French political scientist, Alexis de Tocqueville, in *Democracy in America*, "does not consist in being more enlightened than other nations, but in being able to repair the faults they commit" (294). What Tocqueville is referring to is the ability of the American people to bear witness to the errors of their ways and self-correct. Although Tocqueville's comment appeared in a book first published in 1836, what he noted of Americans still rings true today. It can be observed in the way parents have attempted to navigate school closures.

In a democracy, citizens can elect to either vote by ballot or with their feet. Some parents of school-aged children are opting for the latter. "America's public schools have lost at least 1.2 million students since 2020" reports Shawn Hubler for *The New York Times* in her May 17, 2022, article "With Plunging Enrollment, a 'Seismic Hit' to Public Schools." Although the hit to enrollment predates COVID-19, Hubler says "the coronavirus crisis supercharged that drop." She goes on to explain that experts offer two reasons for the decline. First, parents who were "fed up with remote instructions or mask mandates" have opted to either homeschool or send their children to private schools for in-person classes. Second, "pandemic-related job losses, homelessness and school closures" caused some children to quit school altogether.

But deeper in the article there is another explanation. Some families, according to Hubler, packed up their belongings and moved to states "where schools eschewed remote instructions." One such state was Florida, where Hubler notes that "enrollment has not only rebounded but remains robust." The article is replete with testimonies from families who opted to leave their homes for the remote chance that their child's education would improve with in-person classes. As if to validate Tocqueville's finding, Americans witnessing a problem with

their public schools took it upon themselves to correct the situation. Given the emotional and educational landscape faced by students in 2021, uprooting for better prospects was sensible.

After surveying the situation, researchers have begun to understand the reality observed by Coombs at the onset of the pandemic. From January to June of 2021, the Centers for Disease Control and Prevention (CDC) worked to assess the impact of COVID-19 lockdowns on adolescent behavior (Jones et al.; Krause et al.). The results, published in the April 2022 *Morbidity and Mortality Weekly Report* (MMWR), are disheartening. 66% of "students found it more difficult to complete their schoolwork" (Krause et al. 28) and 44.2% of adolescents to experience "persistent feeling of sadness or hopelessness" (Jones et al. 16). More concerning is that "19.9% had seriously considered attempting suicide and 9.0% had attempted suicide" (Jones et al. 16). To slow the spread of COVID-19, it was understood that limiting in-person interactions generally would reduce infection. However, if the self-reported experiences in the CDC's reports are correct, lockdowns cost students both emotionally and educationally, an expense paid by low-income families.

School closures are estimated to reduce the long-term earning potential of low-income families. The pandemic has exacerbated systemic inequality, reports Mike Cummings in his January 15, 2021, *Yale News* article titled "COVID School Closures Most Harm Students from Poorest Neighborhoods." Throughout the article, Cummings cites a study from Yale economist Fabrizio Zilibotti, who found that students from the lowest-income families are likely to experience "a decline of about a half a point" to their GPA, which will cost them an estimated 25% of their post-educational earning potential (Cummings). Conversely, says Cummings, "researchers found no learning losses" in children from high-income families. He notes that children living in top-income earning communities benefited from contact with "similarly

advantaged peers during school closures" (Cummings). It turns out that school closures led to socioeconomic segregation that is projected to widen the income gap. But access to peer group only explains part of the educational loss.

Some school districts in low-income communities could not facilitate remote learning. In the journal *Current Problems in Pediatric and Adolescent Healthcare*, Charles Oberg and colleagues found only 60% of school districts in low-income communities were able to provide access to remote learning devices (7). They note that students from low-income families already experienced an educational "achievement gap" prior to the pandemic and that school closures are expected to increase that gap by 15-20% (Oberg et al. 7). In other words, school closures deepened the educational divide between the wealthiest and the poorest families. These unequal outcomes might have been acceptable if mitigation strategies made a difference to public health.

However, according to David Leonhardt, senior writer for *The New York Times*, "They haven't made a big difference." In an article titled "Do Covid Precautions Work?" Leonhardt takes a predominately political approach to answering the question, arguing that Democrats are far more likely than Republicans to observe mitigation strategies like wearing masks and closing classrooms. Despite these behavioral differences, Leonhardt points to data gathered by *The New York Times* indicating "the number of COVID cases has recently been somewhat *higher* in heavily Democratic areas." Leonhardt then explores vaccination and mortality differences between Democrats and Republicans, which indicate that residents from counties won by Donald Trump experienced death rates "twice as high" as those from counties won by Joe Biden. The lesson to be learned, says Leonhardt, is "interventions other than vaccination – like masking and distancing – are less powerful than we might wish."

As masking and distancing mandates cease to be enforced, pointing out the associated psychological and sociological disruptions they induce is becoming less conspiratorial and more observational. In time, it may be possible to assess the pandemic without invoking the political affiliation of those who died and instead focus on what can be understood going forward. For the time being, it is important to understand that Leonhardt is wrong. COVID-19 "interventions other than vaccines" *are* powerful ("DO Covid"). School closures have contributed to a mental health crisis and increased systemic inequity between the wealthiest and poorest. What is worse, none of this appears to have been necessary.

Experts found out early on in the pandemic that children were a low-risk group, and the subsequent evidence has only confirmed their findings. In August of 2020, Swedish epidemiologist Jonas Ludvigson wrote in *Acta Paediatrica* that "children are unlikely to be the main drivers of the pandemic" and that opening schools is "unlikely to impact COVID-19 mortality rates in older people" (1525). This comports with a May 2021 study published in the *Journal of School Health*, where researchers examined a sample of more than 3,700 students from two schools with a combined total of 137 confirmed cases of COVID-19 (Gillespie et al. 347). They concluded the majority of cases were asymptomatic, none of the children required hospitalization, and that "91% [of infections] were not associated with school-based transmission" (347). More recently, in January 2022, after conducting a meta-analysis of 65 scientific papers involving "1,214 children younger than five… with laboratory-confirmed COVID-19 infection," Mejbah Bhuiyan and colleagues wrote in the journal *Vaccine*, only 7% presented symptoms requiring hospital admission (667). In aggregate, the science shows that children are less likely to experience strong symptomatic infection. Least of all are they likely to contract COVID-19 from their fellow classmates. Despite the evidence, many states continued

with lockdowns, which have exacted a far greater toll on mental health than COVID-19 posed physical health of children.

For many children, the toll levied by mitigation is more than they can bear. "Children as Young as 8 Should Be Screened for Anxiety, Experts Recommend: Draft Guidance Underscores Pandemic's Toll on Adolescent Mental Health" reads the title of Brianna Abbott's piece in *The Wall Street Journal*. "America's Kids Earn Disappointing Grades on Nation's Report Card," wrote Lauren Camera in an October 2021 headline for the *U.S. News & World Report*. "Not Good for Learning" concurs David Leonhardt, a senior writer for *The New York Times*, in a May 2022 newsletter. Leonhardt found that "students who stayed home for most of 2020-21 […] lost the equivalent of about 50% of a typical school year's math learning." He points out that these losses were more pronounced in low-income neighborhoods, where "high-poverty schools are […] more likely to have unionized teachers, and some unions lobbied for remote schooling" ("Not Good"). The educational loss noted in the *U.S. News & World Report* article was forecasted by Coombs and echoed by Leonhardt. Requiring children to be screened for anxiety has basis in the surveys conducted by the CDC. The situation might have been different for these children if schools opened earlier.

Quantifying the impact of school closures can be done by comparing the physical and mental health outcomes of youths in Illinois to youths in Florida. Illinois schools closed and remained closed for more than a year (State of Illinois). Florida schools reopened for in-person learning in August of 2020 (Blackmore et al. 437). There are two important metrics to consider. First, school closures seemed to have little influence on deaths. As of May 11, 2022, according to the CDC, 30 children aged 0-17 have succumbed to COVID-19 in Illinois, whereas in Florida, the total is 80 (U.S. National Center for Health Statistics). For context, the U.S. Census Bureau

lists the population of Illinois as 12.67 million, and 21.78 million for Florida. Additionally, the CDC lists all other deaths in the same age group accordingly: Illinois, 2,761; Florida, 5,086 (U.S. Census Bureau). Given the population sizes, it is not beyond reason that Florida would have about double the number of COVID-19 deaths.

Second, the mental health differences are significant. According to Mental Health America, a non-profit organization "dedicated to helping all Americans achieve mental wellness," the number of youths (children 12-17) in 2018 who suffered "at least one major depressive episode" was, in Illinois, 126,000, and in Florida, 180,000 (Hellebuych 17). The number of youths who experienced a "severe major depressive episode" the same year was 82,000 and 125,000, respectively (Hellebuych et al. 19). By 2021, the number of children who experienced "at least one major depressive episode" in Illinois increased by 18.25%, and by 6.1% in Florida (Reinert et al. 25). As for the number of children who experienced a "severe major depressive episode," Illinois saw an increase of 26.83%, whereas Florida saw a 0.8% decrease (Reinert et al. 27). Florida and Illinois represent two ends of the extreme when it comes to reopening schools. Increased symptoms of depression experienced by Illinois youths aligns with findings from the CDC that lockdowns contributed to students reporting "persistent feeling of sadness" (Jones et al. 16). The correlation between the number of COVID-19 deaths and reopening schools is weak. The correlation between keeping schools closed and depressive episodes of children 0-17 is strong.

In order to better navigate institutional closures, Americans must embrace individual choice. In terms of the COVID-19, that means accepting the advice given by NIAID director, Dr. Fauci, in an interview on April 10th, 2022, with *ABC News*, where he said COVID-19 is "not going to be eliminated" and it is up to individuals "to make their calculation of the amount of

risk they want to take" ("Uptick in COVID" 3:32–3:47). Making these kinds of calculations requires empowering individuals with options. That might look like what Dr. Leana Wen describes as "One-way masking" where individuals can mask themselves around others who are unmasked, and they will still be protected (qtd in Anderson Cooper, 0:18-1:00). The point is, Americans today have multiple ways to protect themselves from COVID-19 including vaccines, anti-viral medication, testing, and one-way masking. What Americans need now is access to school choice.

The evidence suggests that, for most children, COVID-19 restrictions have done more harm mentally and developmentally than good in preventing deaths. When the United States closed its schools to children, families felt the burden immediately. Experts have expanded on what parents observed all along, that closures weighed heavy on the wellbeing of schoolchildren. What is more, harmful aspects of lockdowns were not evenly distributed; they hurt disadvantaged communities by increasing the educational and economic gap between the wealthiest and poorest Americans. As mitigation strategies like school closures, masking, and distancing cease to be enforced, the situation has since come back full circle: Americans are once again assessing their own risk tolerance and proceeding forward as they see fit. Although that may seem natural, it only happened after countless changes to circumstances over the past two years. But one thing has not changed: for most, access to alternative education options is still limited.

In order create a more equitable education system, one that allows families of all economic backgrounds to prosper, Americans must embrace alternative options. Simply put, all families, not just the wealthiest, should have the ability to educate around a system they believe is not working for them. Maybe that means a voucher system, where parents can take the public

funds allotted to their child and apply them to an education outside of the public school system. That might look like staying remote or maybe greater access to private schools. Whatever form it takes, the goal should be to increase the options available to all Americans, with respect to their individual risk level. That should include teachers who are themselves at higher risk. If Americans can agree that more options mean better outcomes, then more students will be able to reclaim the education and income potential lost to them during the pandemic. With any luck, the education gap faced by disadvantaged families will close. With all hope, fewer children will be found weeping over tear-soaked laptops as their future prospects diminish.

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